## 15.2 Hyperemesis Gravidarum*

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "I cannot keep even water down."<br>**Objective**<br>• Vomiting more than 5 times a day<br>• Ketones in urine<br>• HR 110/min; dry mouth | **ACTUAL**<br>Inadequate fluid volume related to persistent vomiting as evidenced by ketonuria, tachycardia and dry mucosa. | • Urine free of ketones and output adequate within 24–48 hours. | • Give IV fluids with electrolytes as prescribed.<br>• Give antiemetics as prescribed.<br>• Keep an intake and output chart and test urine for ketones daily.<br>• Give thiamine before any fluid containing dextrose, as ordered. | • Considered met when she is hydrated and ketone-free. |
| **Subjective**<br>• "I have lost weight."<br>**Objective**<br>• Lost 4 kg<br>• Almost nothing eaten for a week | **ACTUAL**<br>Inadequate nutritional intake related to vomiting and reduced intake as evidenced by weight loss. | • Tolerates food by mouth and stops losing weight. | • Bring back small, dry meals gradually.<br>• Refer to the dietitian.<br>• Weigh twice a week. | • Goal reached if weight is stable. |
| **Subjective**<br>*No specific complaint*<br>**Objective**<br>• Prolonged vomiting | **RISK**<br>Risk for impaired water-electrolyte balance as evidenced by risk factors: loss of gastric fluid. | • Potassium and sodium stay normal. | • Check electrolytes daily.<br>• Add replacement to IV fluids as prescribed.<br>• Report muscle weakness or palpitations. | • Met if values remain normal. |

*References for this care plan: [35], [47], [95]. Full entries are listed in the References section of the CPMN book.